

EATING DISORDERS

- Andrea Coladner, D.O.
- Assistant Professor | Family Medicine
- Old Westbury, 2025



NEW YORK INSTITUTE OF TECHNOLOGY

College of Osteopathic
Medicine

SESSION OBJECTIVES

1. Compare and contrast feeding and eating disorders as per Diagnostic and Statistical Manual of Mental Disorders, (DSMV) classification
2. Discuss the common behavioral and psychological features
3. Review the signs, symptoms, risk factors and subtle physical exam findings that may support each diagnosis
4. Utilize BMI calculations to aid in diagnosis of eating disorders
5. Identify common consequences and complications
6. Explain appropriate treatment options

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- A 9 month African American male presents for his well child exam. His mother voices concerns that he always seems to be chewing something. She has checked his mouth and has seen chewed up food. She sometimes hears him sound as if he is vomiting but never sees anything come out of his mouth. This has been occurring for 2 months. He does not have any prior medical problems.

What is his most likely diagnosis?

PICA **A**

Rumination Disorder **B**

Unspecified feeding or
eating disorder **C**

Other specified feeding
or eating disorder **D**

-
- A 25 yo Caucasian female presents for her 30 week OB visit for routine follow up. She tells you that she has been craving and eating things like chalk and dirt and asks if this is normal. She has been feeling this way for 6 weeks.

What is her most likely diagnosis?

Presenter Notes

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Poll Title: What is her most likely
diagnosis?

<https://www.poll Everywhere.com/poll/14295N120>

Rumination
Disorder **A**

Avoidant/restrictive
food intake disorder **B**

Unspecified feeding
or eating disorder **C**

PICA **D**

Eating Disorders- Definition

- Eating disorders are mental illnesses characterized by disturbances in:
 - Body image
 - Weight
 - Dietary patterns

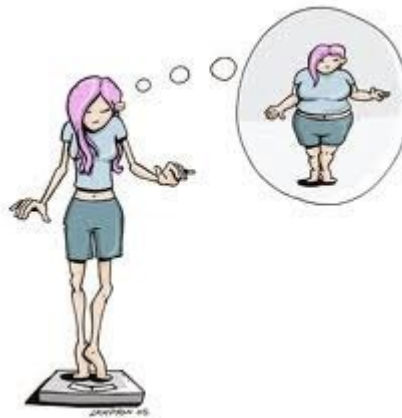


EATING DISORDERS -DSM 5

- The DSM 5 list of eating disorders
 - Anorexia nervosa
 - Bulimia nervosa
 - Binge eating disorder
 - Avoidant/restrictive food intake disorder
 - Pica
 - Rumination disorder
 - Other specified feeding or eating disorder
 - Unspecified feeding or eating disorder

EATING DISORDERS -EPIDEMIOLOGY

- The population most at risk for eating disorders are men and women between 12 and 25 years of age



EATING DISORDERS -SCREENING

- A quick clinical tool that can be used in the office is the **Eating Disorder Screen for Primary Care**
- Are you satisfied with your eating patterns? (No is abnormal)
- Do you ever eat in secret? (Yes is abnormal)
- Does your weight affect the way you feel about yourself? (Yes is abnormal)
- Have any members of your family suffered with an eating disorder? (Yes is abnormal)
- Do you currently suffer with or have you ever suffered in the past with an eating disorder? (Yes is abnormal)

EATING DISORDERS -SCREENING

- The **Eating Attitudes Test (EAT)**
 - Most widely used
 - 26-item self reported questionnaire
 - Accuracy rate of at least 90 percent when screening patients for the presence of an eating disorder
 - Cutoff score of 20

ANOREXIA NERVOSA



ANOREXIA NERVOSA (AN)-DIAGNOSIS

- For diagnosis of AN the DSM V requires each of the following for a period of at least 3 months
 - Restriction of energy intake that leads to a low body weight (BMI<18.5)
 - Based on patient's age, sex, developmental trajectory, and physical health
 - Intense fear of gaining weight or becoming fat or persistent behavior that prevents weight gain
 - Despite being underweight
 - Distorted perception of body weight and shape ,extreme influence of weight and shape on self-worth or denial of the medical seriousness of one's low body weight

ANOREXIA NERVOSA (AN)-BMI

- Low body weight
 - BMI < 18.5
- Underweight – <18.5 kg/m²
- Normal weight – ≥18.5 to 24.9 kg/m²
- Overweight – ≥25.0 to 29.9 kg/m²
- Obesity – ≥30 kg/m²
- BMI = body weight (in kg) ÷ height (in meters) squared
- Though widely used, the BMI is not the best measure of obesity as it does not account for fat vs muscle weight

ANOREXIA NERVOSA (AN)

- 2 Subtypes
 - **Restricting***-weight loss through:
 - Dieting
 - Fasting
 - Excessive exercise
 - No recurrent bingeing and purging
 - **Binging and Purging***

ANOREXIA NERVOSA (AN)

- Prevalence
 - Women 3 times more likely than men
- The median age is 18 years



EATING DISORDERS

- Classified as mental illness but also have medical consequences
- Anorexia nervosa and Bulimia nervosa are among mental disorders associated with the highest mortality risk



ANOREXIA NERVOSA (AN)-SYMPTOMS

- Most common medical symptoms
 - Amenorrhea
 - Exertional fatigue
 - Weakness
 - Cold intolerance
 - Palpitations
 - Dizziness
 - Abdominal pain and bloating
 - Early satiety
 - Constipation
 - Swelling of the feet



ANOREXIA NERVOSA (AN)-SIGNS

- Common Signs
 - Low body mass index ($<17.5 \text{ kg/m}^2$)*
 - Emaciation (body weight less than 70 percent of ideal body weight)
 - Hypothermia (core temperature $<35^\circ\text{C}$ or 95°F)
 - Bradycardia (pulse <60 beats per minute)
 - Hypotension (systolic blood pressure $<90 \text{ mmHg}$ and/or a diastolic blood pressure $<50 \text{ mmHg}$)
 - Hypoactive bowel sounds
 - Xerosis (dry, scaly skin)
 - Brittle hair and hair loss
 - Lanugo hair growth (soft fine hair)
 - Abdominal distention

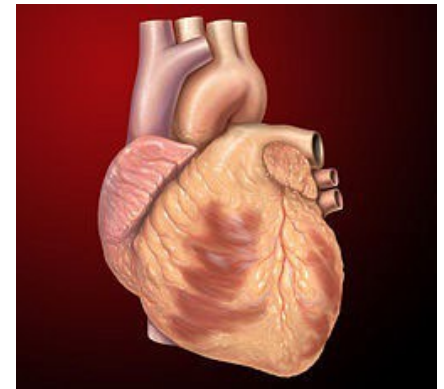
ANOREXIA NERVOSA (AN)-COMPLICATIONS

- Complications
 - Due to malnutrition and weight loss
 - Starvation → protein and fat catabolism → loss of cellular volume and atrophy of the heart, brain, liver, intestines, kidneys, and muscles



ANOREXIA NERVOSA (AN)- COMPLICATIONS

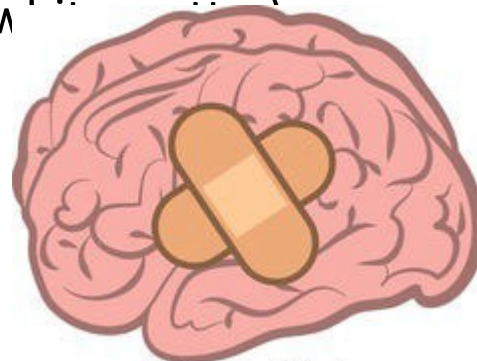
- Cardiovascular
 - Myocardial atrophy
 - Mitral valve prolapse
 - Pericardial effusion
 - Bradycardia
 - Arrhythmia, which may cause sudden death
 - Electrocardiogram (ECG) changes
 - Hypotension
 - Acrocyanosis



ANOREXIA NERVOSA (AN)- COMPLICATIONS

- Neurologic

- Cerebral atrophy (decreased gray and white matter)
- Enlarged ventricles
- Cognitive impairment
- Peripheral neuropathy
- Seizures



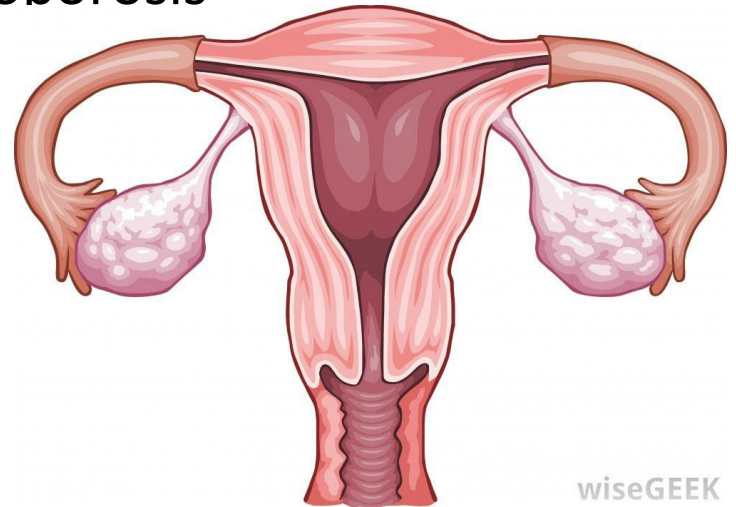
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Anorexia Nervosa (AN)- Complications

- Endocrine
- Hypothalamic dysfunction
- Thermoregulation problems
 - Satiety
 - Sleep
 - Autonomic cardiorespiratory imbalance (orthostasis)
- Osteoporosis and pathologic stress fractures
- Euthyroid sick syndrome
- Hypercortisolemia
- Hypoglycemia

ANOREXIA NERVOSA (AN)- COMPLICATIONS

- Reproductive
 - Reduced gonadal stimulation
 - Amenorrhea (females)
 - Causes decreased ovarian function and estrogen on bones which can lead to osteoporosis
 - Erectile dysfunction (males)



ANOREXIA NERVOSA (AN)-COMPLICATIONS

- Refeeding Syndrome
- A potentially fatal condition caused by a rapid drop in serum phosphorous, magnesium, and potassium
 - Occurs with excessive reintroduction of calories
 - Particularly carbohydrates
- Associated with acute heart failure and neurologic symptoms



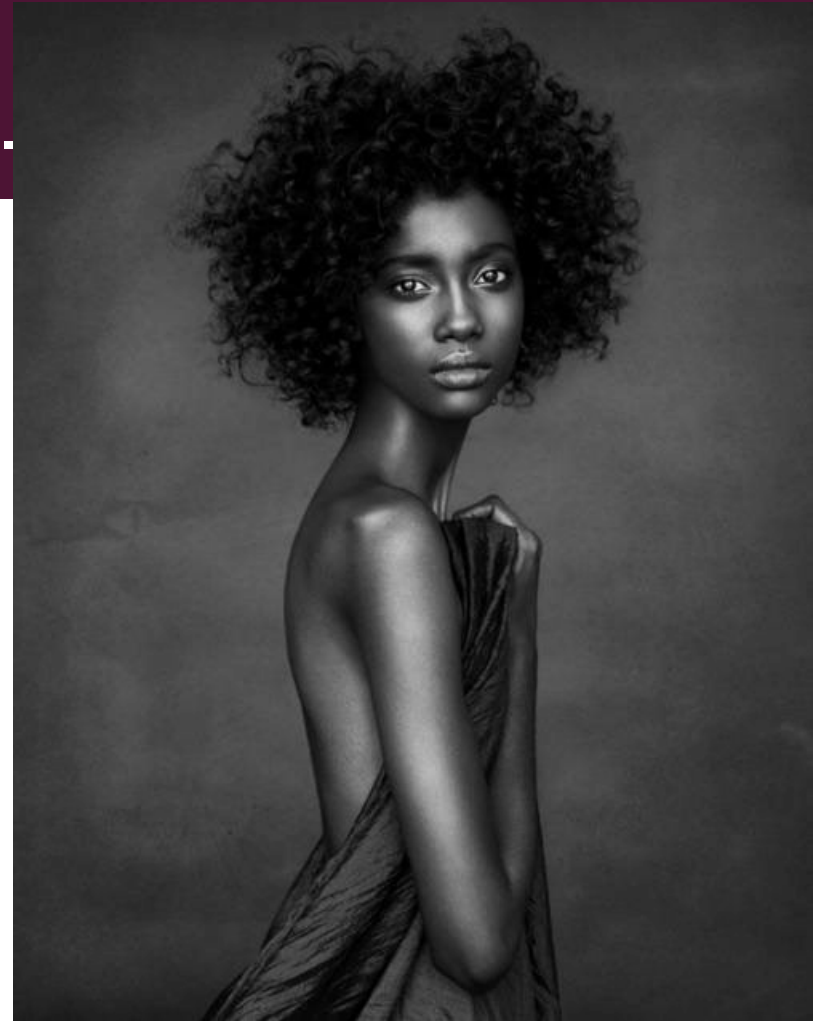
ANOREXIA NERVOSA (AN)-COMPLICATIONS

- Common causes of death
 - Suicide
 - Tachyarrhythmias



ANOREXIA NERVOSA (AN).

- Treatment
 - Nutritional Rehabilitation
 - Psychotherapy



BULIMIA NERVOSA



BULIMIA NERVOSA (BN)- DIAGNOSIS

- Diagnosis
 - Episodes of binge eating
 - Inappropriate compensatory behavior to prevent weight gain.
 - Binge eating and inappropriate compensatory behaviors occur, on average, at least once a week for three months.
 - The patient's self-evaluation is unduly influenced by body shape and weight.
 - The disturbance does not occur exclusively during episodes of anorexia nervosa
 - People can fall within the normal range for their weight

BULIMIA NERVOSA (BN)-DIAGNOSIS

- Inappropriate compensatory behavior
 - Self induced vomiting
 - Fasting
 - Misuse of laxatives, diuretics or other meds
 - Excessive exercising



BULIMIA NERVOSA (BN)-

- Binge Eating
 - Eating an unusually large amount of food in a discrete period of time (eg. 2 hours)
 - Patients feel that the eating cannot be controlled during the episode
 - Equivalent of 2 meals (2000 kcal)
 - Continues until the patient is painfully full or uncomfortable



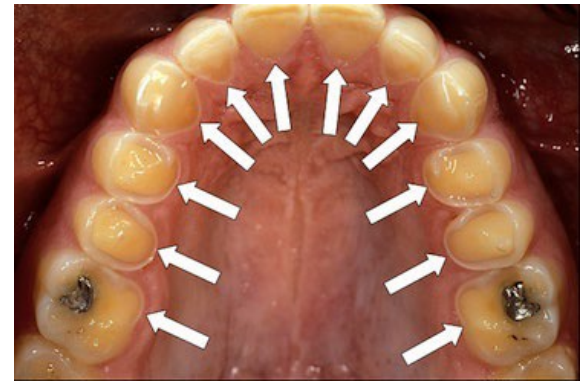
BULIMIA NERVOSA (BN)

- Prevalence
 - Women 5 times more likely than men
- The median age of onset is 18 years



BULIMIA NERVOSA (BN)-SIGNS

- Common Signs
 - Tachycardia
 - Hypotension (<90 mmHg systolic)
 - Xerosis (dry skin)
 - Parotid gland swelling (sialadenosis)
 - Erosion of dental enamel



BULIMIA NERVOSA (BN)-COMPLICATIONS

- Medical Complications
 - Dehydration
 - Hypokalemia
 - Menstrual irregularities
 - Mallory-Weiss syndrome
 - Ipecac-induced myopathy (skeletal, cardiac)
 - Erosion of dental enamel

BULIMIA NERVOSA (BN)-TREATMENT

- Treatment
 - Nutritional rehabilitation
 - Psychotherapy
 - Pharmacotherapy
 - Monitoring patients for medical complications



BULIMIA NERVOSA (BN)-TREATMENT

- First-line
 - Fluoxetine (SSRI)
- Second-line
 - different SSRI (eg, citalopram, fluvoxamine, and sertraline)
- Third-line treatment
 - tricyclic antidepressant



BINGE EATING



BINGE EATING

- **Binge eating disorder** is characterized by recurrent binge eating episodes during which a person feels a loss of control and marked distress over his or her eating. Unlike bulimia nervosa, binge eating episodes are not followed by purging, excessive exercise or fasting. As a result, **people with binge eating disorder often are overweight or obese.**

BINGE EATING DISORDER-DIAGNOSIS

- Diagnosis
 - Episodes of binge eating
- Binge eating episodes have at least three of the following:
 - Eating more rapidly than normal
 - Eating until feeling uncomfortably full
 - Eating large amounts of food when not feeling physically hungry
 - Eating alone because of embarrassment by the amount of food consumed
 - Feeling disgusted with oneself, depressed, or guilty after overeating
- Episodes occur, on average, at least once a week for three months.
- No regular use of inappropriate compensatory behaviors (eg, purging, fasting, or excessive exercise) as are seen in bulimia nervosa.
- Binge eating does not occur solely during the course of bulimia nervosa or anorexia nervosa.

BINGE EATING DISORDER

- More prevalent than Anorexia or Bulimia
- Median age of onset is 21 years old
- Approximately 50 percent of individuals in the community with binge eating disorder are overweight or obese
- Female:Male is 2:1



BINGE EATING DISORDER

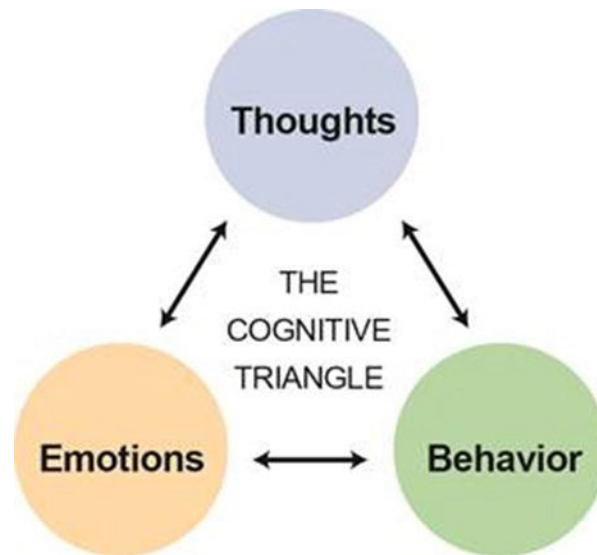
- At increased risk to develop comorbid general medical disorders
 - Chronic pain
 - Diabetes mellitus
 - Hypertension
- Commonly suffer comorbid psychopathology (most likely to be anxiety)

BINGE EATING DISORDER- TREATMENT

- Treatment
 - Psychotherapy
 - Cognitive Behavioral Therapy

COGNITIVE BEHAVIORAL THERAPY (CBT)

- Evidence-based treatment for psychiatric disorders
- Uses thought exercises or real experiences to facilitate symptom reduction and improved functioning



COGNITIVE BEHAVIORAL THERAPY (CBT)

Includes:

- Education
- Relaxation Exercises
- Coping skills training
- Stress management or assertiveness training



AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER



AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Onset usually occurs in infancy or early childhood and may persist into adulthood



AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Diagnosis
 - The eating behavior leads to a persistent failure to meet nutritional and/or energy needs, manifested by at least one of the following:
 - Clinically significant weight loss, or in children, poor growth or failure to achieve expected weight gain
 - Nutritional deficiency
 - Supplementary enteral feeding or oral nutritional supplements are required to provide adequate intake
 - Impaired psychosocial functioning

AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Diagnosis Continued
 - The eating or feeding disturbance is not due to lack of food or a cultural practice.
 - The disturbance does not occur solely in the course of anorexia nervosa or bulimia nervosa, and body weight and shape are not distorted.
 - The disturbance is not due to a general medical condition or another mental disorder. When avoidant/restrictive food intake disorder occurs in the context of another illness, the eating disturbance is both out of proportion to what is expected for the other illness and warrants additional clinical attention.

PICA



PICA

- Onset
 - Usually during childhood, but can occur during adolescence or adulthood
- May be a clinical manifestation of iron deficiency anemia



PICA

- Diagnosis
 - Repeated eating of nonfood substances that are not nutritional, for at least one month.
 - The eating behavior is inappropriate to the patient's developmental level, and is not culturally supported or socially normal.
 - Minimum age of 2 years is suggested
- If the eating behavior occurs in the context of another mental disorder or general medical condition the severity of the eating behavior warrants additional clinical attention.

PICA

- Pregnant women are most likely group affected by PICA followed by school aged children



PICA

- Treatment
 - Combined behavioral, social, and medical approaches



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PICA

- Diagnosis
 - Repeated eating of **nonfood substances** that are not nutritional, for at least **one month**.
 - The eating behavior is **inappropriate to the patient's developmental level**, and is **not culturally supported or socially normal**.
 - Minimum age of 2 years is suggested
 - If the eating behavior occurs in the context of another mental disorder or general medical condition the severity of the eating behavior warrants additional clinical attention.

RUMINATION DISORDER



RUMINATION DISORDER

- In healthy children, this disorder usually appears between ages 3 months and 12 months
- The disorder can have an episodic course or occur continuously

RUMINATION DISORDER-DIAGNOSIS

- Repeated regurgitation of food, which may be rechewed, reswallowed, or spit out
 - the eating disturbance occurs for at least one month.
- Regurgitation of food is not due to a general medical condition, such as gastroesophageal reflux disease or pyloric stenosis.
- Regurgitation does not occur solely during the course of avoidant/restrictive food intake disorder, anorexia nervosa, binge eating disorder, or bulimia nervosa.
- If the eating behavior occurs in the context of another mental disorder or general medical condition, the severity of the eating behavior warrants additional clinical attention.

RUMINATION DISORDER TREATMENT

- Behavioral Treatment
 - Diaphragmatic breathing
 - Gum chewing after meals



EATING DISORDER BRICK

- <https://exchange.scholarrx.com/brick/eating-and-feeding-disorders>

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- A 9 month African American male present for his well child exam. His mother voices concerns that he always seem to be chewing something. She has checked his mouth and has seen chewed up food. She sometimes hears him sound like he vomiting but never sees anything come out of his mouth. This has been occurring for 2 months. He does not have any prior medical problems.

-
- A 9 month African American male present for his well child exam. His mother voices concerns that he **always seem to be chewing something**. She has checked his mouth and has seen **chewed up food**. She sometimes hears him **sound like he vomiting** but never sees anything come out of his mouth. This has been **occurring for 2 months**. He does not have any prior medical problems.

RUMINATION DISORDER-DIAGNOSIS

- Repeated regurgitation of food, which may be rechewed, reswallowed, or spit out
 - the eating disturbance occurs for at least one month.
- Regurgitation of food is not due to a general medical condition, such as gastroesophageal reflux disease or pyloric stenosis.
- Regurgitation does not occur solely during the course of avoidant/restrictive food intake disorder, anorexia nervosa, binge eating disorder, or bulimia nervosa.
- If the eating behavior occurs in the context of another mental disorder or general medical condition ,the severity of the eating behavior warrants additional clinical attention.

OTHER MATERIAL TO REVIEW

- Complete list of medical complications of anorexia nervosa

[https://www.uptodate.com/contents/image?imageKey=PSYCH%2F67080&topicKey=PSYCH%2F15777&source=see link](https://www.uptodate.com/contents/image?imageKey=PSYCH%2F67080&topicKey=PSYCH%2F15777&source=see_link)

- References

NIMH (National Institute of
Mental Health) website
DMS – 5

Core References:

Clinical Key -National Institute for Health and Care Excellence (NICE)

Debra K. Katzman, Sarah A. Kearney and Anne E. Becker Sleisenger and Fordtran's Gastrointestinal and Liver Disease, Chapter 9, 130-147.e7

Emily R. Katz, Robert L. Kitts and David R. DeMaso. Nelson Textbook of Pediatrics, Chapter 23,

Up to Date

Mehler PS, Brown C. Anorexia nervosa – medical complications. Journal of Eating Disorders. 2015;3:11. doi:10.1186/s40337-015-0040-8.



Lecture Feedback Form:

[https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448F
WYXREL4R](https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R)